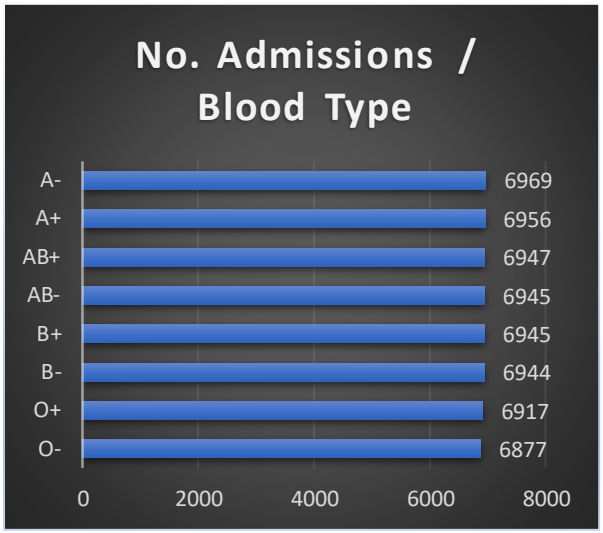
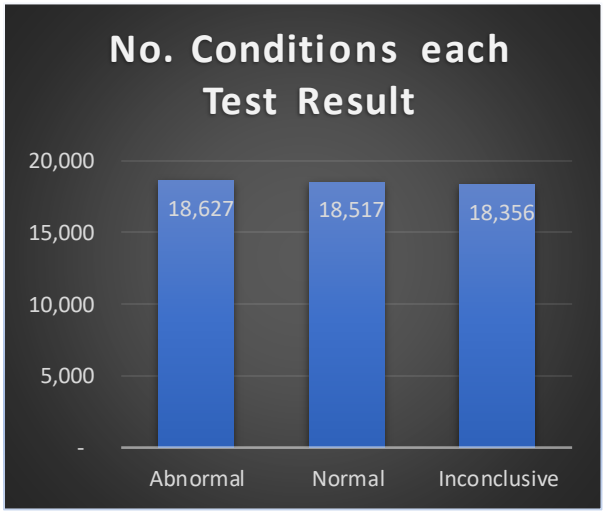
Prepared by: Omar Hussein
Data Analyst

Financial Report

1. The data indicates that gender and blood type do not directly affect the number of cases, as they follow a random pattern both overall and for each individual disease mentioned in the data. There are no significant differences in the number of cases for any particular blood type.



2. The data indicates that the medication used does not directly affect the outcome of the clinical test. The rate of normal, positive, and abnormal results is almost equal across all types of treatments and for all diseases included in the data. This suggests that the medications used are effective but lack the specificity needed to raise the recovery rate to higher levels.



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3.

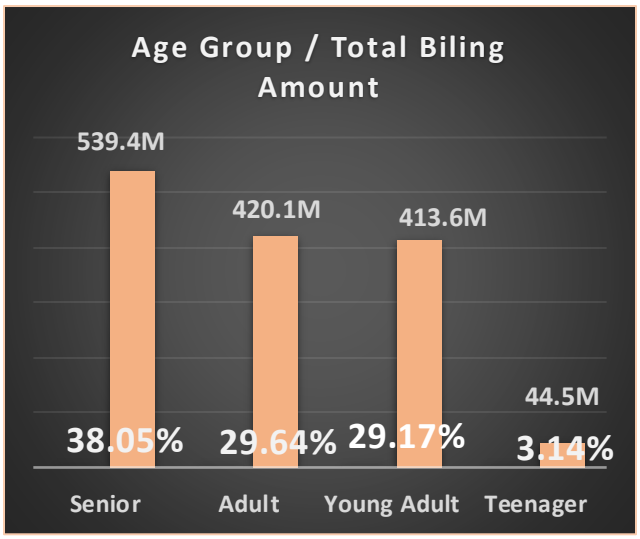
- The data indicates that age significantly affects the overall case rate. The Seniors category is the highest, significantly outnumbering the other categories.

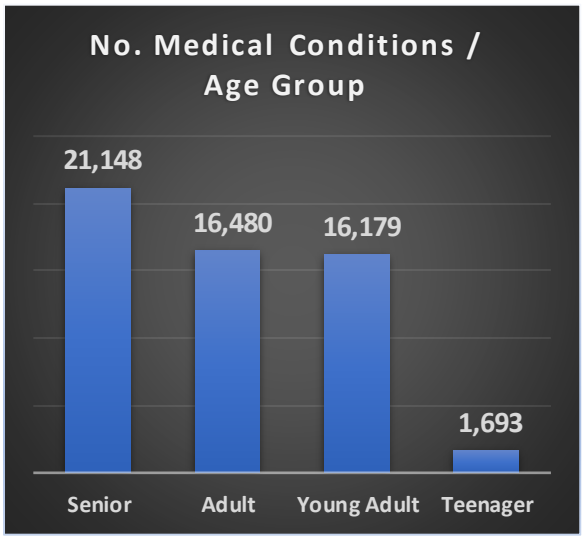
- The Adults and Young Adults categories occupy a relatively middle position, with the Seniors category having approximately 4,668 more cases than them, representing an increase of about 28% overall.

- The Teenagers category is the lowest, representing less than 10% of the total. The overall total of cases confirms that this group is the least susceptible to the diseases identified in the data.

- The largest budget is allocated to the Seniors category, with approximately 38% of the total budget spent by insurance companies going to this group. This is followed by the Adult and Young Adult categories, each receiving about 30%. The Teenager category receives only 3.4% of the total budget, which is proportional to the number of cases in each category.

- The distribution of the budget spent by each category indicates that the average budget for each category is almost constant, both overall and across all types of illnesses and cases. (It was noted that the Teenager category, despite its small size, often receives a slightly higher average budget.) This suggests a relatively fixed pricing policy based on the type of medical procedure, regardless of age. It also indicates a lack of prioritization for the elderly in the budget.





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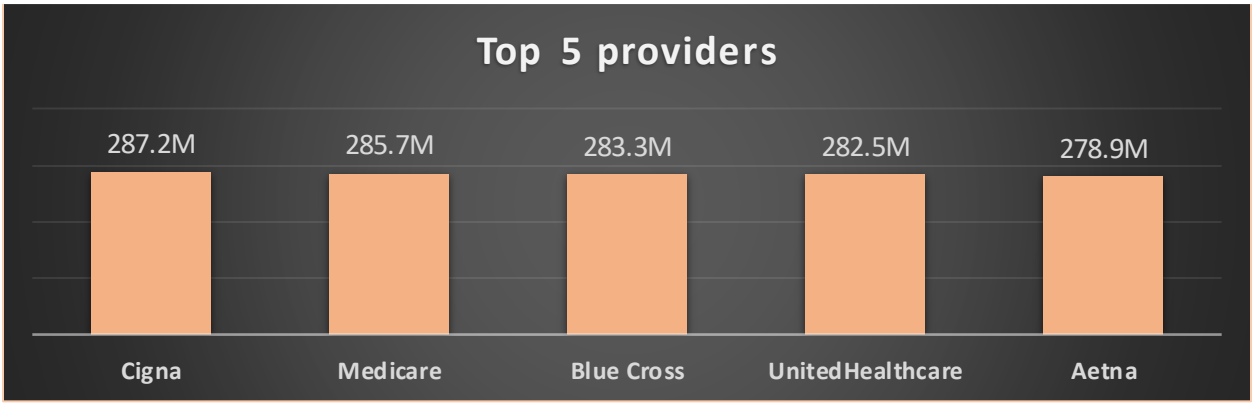
Recommendation:

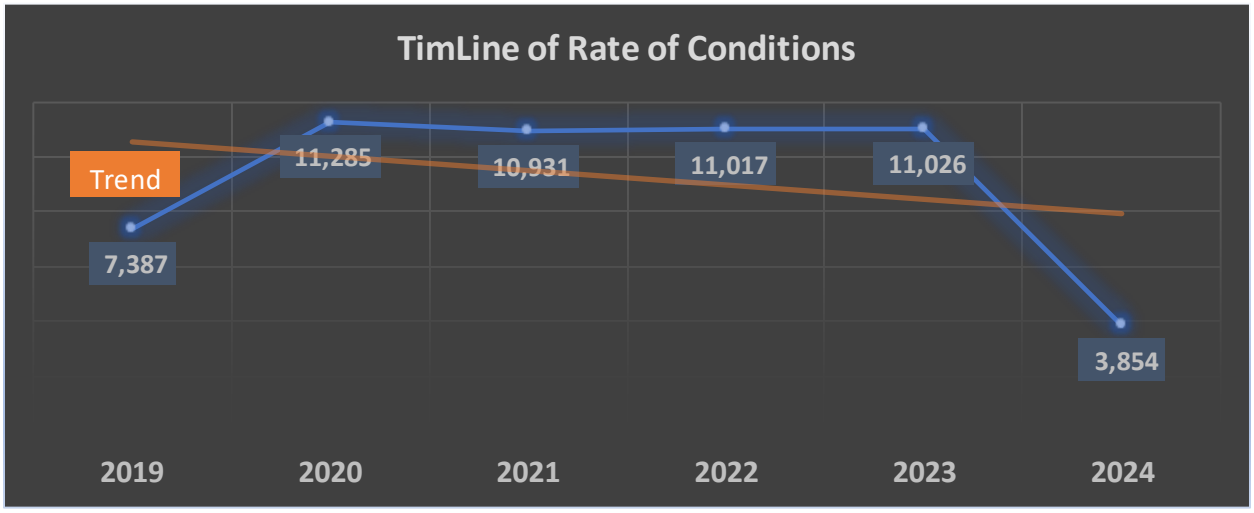
- Implement regular screening programs for the elderly in the community.
- Provide educational guidance to the elderly and adults on adopting healthy diets, routine activities, and natural remedies that contribute to reducing the risk of common diseases such as diabetes, cancer, and others.
- Recommendation: Re-evaluate the pricing structure to consider age, particularly the complexity of the patient's medical condition, to avoid future losses in this group due to the fixed budget's inability to cover treatment costs for elderly patients.

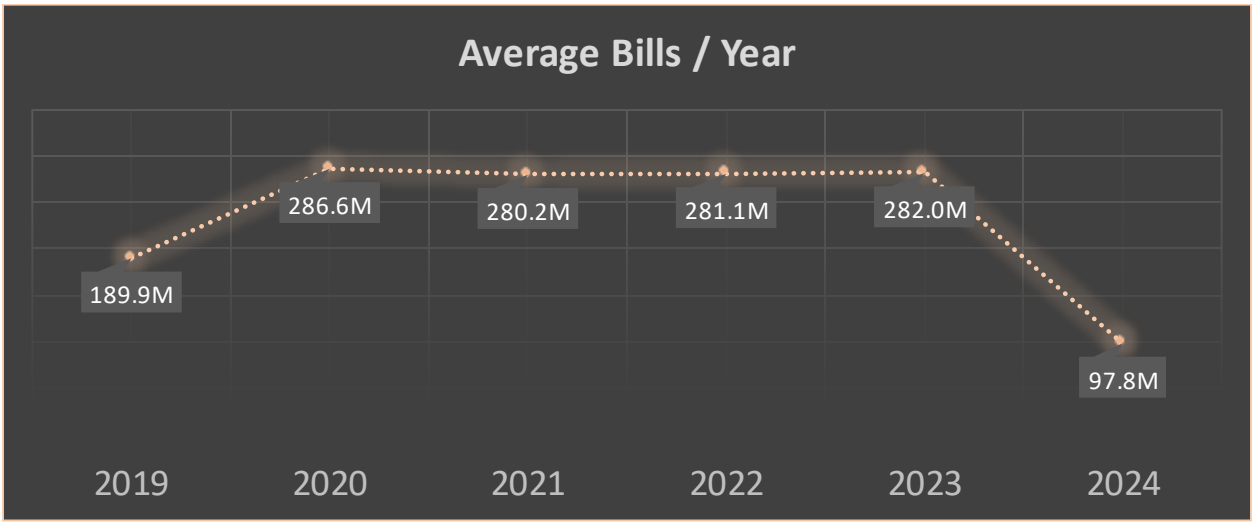
4. The budget is distributed relatively fairly among the five major insurance companies, indicating the hospital's diversification and reduced financial risk.



5.

- The timeline of case rates indicates a significant increase from the beginning of 2019 until the end of the year, with an increase of approximately 3,600 cases during that year.
- The period from the beginning of 2020 to the end of 2023 witnessed a general stabilization in the rate.
- This stability was followed by a sharp decline until May 2024, indicating the presence of an external factor or success factor that led to this decrease of approximately 4,000 cases compared to the previous year.
- The timeline for both case rates and expenditure shows a correlation between any increase or decrease in total expenditure and the total number of cases. This is a strong statistical indicator suggesting a stable average cost per case per year and confirming insurance companies' commitment to paying the amounts due per case, while hospitals do not artificially inflate prices during peak periods.





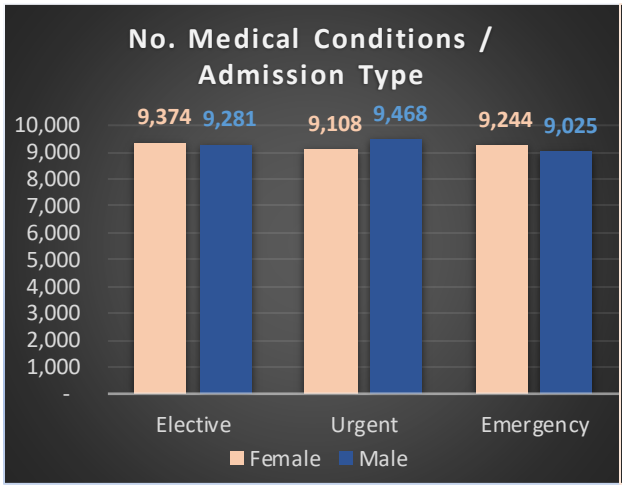
Recommendation:

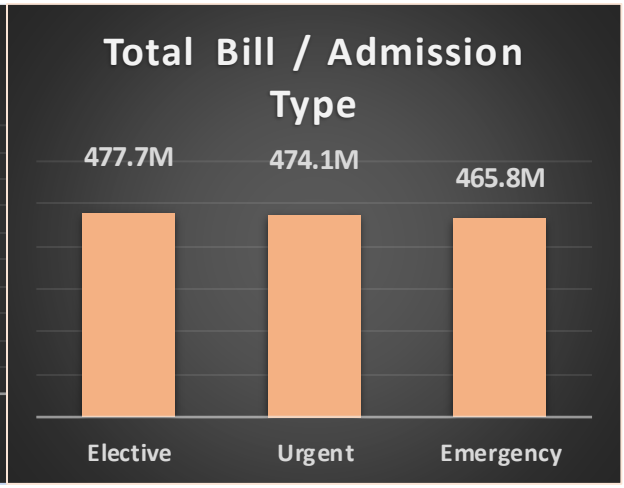
Develop a systematic plan to study the most prominent contributing factors and attempt to implement replicable factors to achieve the highest possible rate of reduction in the coming years.

6.

- Data indicates that the overall admission rate (emergency, urgent, elective) is almost equal at the national level, necessitating hospitals to be fully prepared to receive emergency and urgent cases, with double the readiness for regular admissions. The relatively high rates of emergency and urgent admissions are considered a dangerous trend.

The total income spent on each type of case is almost equal, around $470 million. This is a natural indicator due to the similar number of cases per type.





Recommendation:

Work to raise public awareness about undergoing regular medical checkups to ensure constant preparedness for treating and preventing any disease in its early stages before it reaches a critical stage.